FAX

To: **USER-ENTERED FAX**
Company:
Fax: 520-325-4227
Phone:

From: **Laura Guzman**
Fax:
Phone:
E-mail:

NOTES:

Date and time of transmission: Thursday, May 7, 2026 3:43:00 PM
Number of pages including this cover sheet: 19

EL RIO HEALTH

839 W. Congress St
Tucson, Az 85745

Date: May 7, 2026

To:
Address:1040 S. Harrison Rd Ste 100
Phone:520-357-1174

Referral From:
El Rio Health
Fax: 520-309-2560

Patient Name: Paige Molinari

Date Of Birth: 12/13/1993

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ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F

Referral: Therapy - From Michelle Baas, CNM to BodyCentral Physical Therapy - Harrison (Authorized)



Paige Molinari

Legal: Paige Molinari
32 y.o. female (DOB: 12/13/1993)

Pt Class
Service
Location: El Rio Grant and Dodge
GD FAMILY MED

Referring Dx: No info available

Patient Demographics

18775 S Sonoita Hwy
Vail AZ 85641-9108

PCP: Maya Adams, CNM
Language: English
Phone: 631-896-3452

Emergency Contacts

Cody Nelson (Spouse) - 321-746-1343
-

Guarantor

MOLINARI,PAIGE

Relation: Self
Address: 18775 S SONOITA HWY
Vail, AZ 85641-9108

Home: 631-896-3452
Work
Mobile
Employed at: BENSON HOSPITAL (Full Time)

Coverages

TRIWEST HA VA COMMUNITY

Subscriber: MOLINARI,PAIGE
Relation: Self
Sub ID #: 1469131650
Group #

- No Secondary Coverage -

Subscriber
Relation
Sub ID #
Group #

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F

Referral: Therapy - From Michelle Baas, CNM to BodyCentral Physical Therapy - Harrison (Authorized) (continued)

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F

Referral: Therapy - From Michelle Baas, CNM to BodyCentral Physical Therapy - Harrison (Authorized) (continued)

Referral # 1044489

Priority:Routine

Patient

Name: Paige Molinari(Paige Molinari)
Address: 18775 S Sonoita Hwy
Vail AZ 85641-9108

Patient Phone: 631-896-3452
DOB: 12/13/1993

Referred By Information

Provider: Michelle Baas,CNM
Electronically Signed on: 4/15/2026
Provider NPI: No NPI Available
Phone: 520-670-3909
Fax: 520-309-2560

Department: SE MIDWIFERY
Address: 6950 E GOLF LINKS RD
TUCSON AZ 85730-1017

Coverage Information

Primary Payer/ Plan
Payer: TRIWEST HA VA COMMUNITY CARE
Subscriber ID: 1469131650
Effective Date: 08/19/24
Auth #:

Secondary Payer/ Plan
Payer: No secondary coverage
Subscriber ID:
Effective Date:
Auth #:

Referred To Information

Loc/ Provider: BodyCentral Physical Therapy - Harrison
Address: 1040 S. Harrison Rd Ste 100-Tucson, AZ 85748

Phone: 520-357-1174
Fax: 520-325-4227

Referral Information

Status: Authorized - No Prior Authorization Required
Reason: Consult and Treat
Start Date: 04/15/2026 End Date: 04/15/2027
Authorized Visits: 1

Procedures and Diagnoses

Procedures:
REFERRAL TO PHYSICAL THERAPY
My clinical question is: Patient is 6 weeks PP, noticing some pelvic floor dysfunction and specifically the stream of urine is flowing side ways and not directly downward into the toilet bowl at times. Needs PFPT

Diagnoses:
M62.89 (ICD-10-CM) - Pelvic floor dysfunction in female

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ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F

Referral: Therapy - From Michelle Baas, CNM to BodyCentral Physical Therapy - Harrison (Authorized) (continued)

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Referral Info

Therapy #1044489

Reason: Consult and Treat
Class: Outgoing
Status updated on: 5/7/2026
Priority: Routine
Status: Authorized - No Prior Authorization Required
Valid dates: From 4/15/2026 to 4/15/2027

Referred From

Location: ELR Southeast
Department phone: 520-670-3909
Provider phone: 520-670-3909
Department: SE MIDWIFERY
Provider: Michelle Baas, CNM
Provider address: 839 W Congress St Tucson AZ 85745

Referred To

Location: BodyCentral Physical Therapy - Harrison
Specialty: Physical Therapy

Visits

Requested	Authorized	Completed	Scheduled
1	1	0	0

Procedures

REF87 - Referral to Physical Therapy (El Rio)

Number requested: 1
Number approved: 1

Diagnoses

- M62.89 (ICD-10-CM) - Pelvic floor dysfunction in female

Coverages

TRIWEST HA VA COMMUNITY CARE

Plan: TRIWEST HA VA COMMUNITY CARE
Covered: Covered
From: 8/19/2024
Member #: 1469131650

Order

Referral to Physical Therapy (El Rio)

Electronically signed by: **Michelle Baas, CNM on 04/15/26 0932**
Status: **Active**
Ordering user: Michelle Baas, CNM 04/15/26 0932
Authorized by: Michelle Baas, CNM
Frequency: Routine 04/15/26 -
Quantity: 1
Diagnoses
Pelvic floor dysfunction in female [M62.89]
Ordering provider: Michelle Baas, CNM
Ordering mode: Standard
Class: Internal Referral

Questionnaire

Question	Answer
My clinical question is:	Patient is 6 weeks PP, noticing some pelvic floor dysfunction and specifically the stream of urine is flowing side ways and not directly downward into the toilet bowl at times. Needs PFPT

Patient

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F

Patient (continued)

Problem List as of 5/7/2026

Problems last reviewed by Michelle Baas, CNM on 4/15/2026 0922

Elevated glucose level RESOLVED

Diagnosis: Elevated glucose level
Noted on: 12/17/2025
Priority: Medium
Chronic: No

History of pre-eclampsia in prior pregnancy, currently pregnant in third trimester

Diagnosis: History of pre-eclampsia in prior pregnancy, currently pregnant in third trimester
Noted on: 02/19/2025
Priority: Medium
Chronic: No

Mother currently breast-feeding (CMS/HCC)

Diagnosis: Mother currently breast-feeding (CMS/HCC)
Noted on: 09/15/2025
Priority: Medium
Chronic: No

Obesity in pregnancy, antepartum

Diagnosis: Obesity in pregnancy, antepartum
Noted on: 08/18/2025
Priority: Medium
Chronic: No

Short interval between pregnancies affecting pregnancy, antepartum

Diagnosis: Short interval between pregnancies affecting pregnancy, antepartum
Noted on: 08/17/2025
Priority: Medium
Chronic: No

Current Medications

Medications

This report is for documentation purposes only. The patient should not follow medication instructions within. For accurate instructions regarding medications, the patient should instead consult their physician or after visit summary.

Current Medications

ibuprofen 600 MG tablet

Instructions: TAKE ONE TABLET BY MOUTH FOUR TIMES A DAY AS NEEDED TAKE IN THE MORNING, AT NOON, IN THE EVENING, AND AT BEDTIME FOR MILD PAIN
Authorized by: Elya Showalter, CNM
Ordered on: 3/20/2026
Start date: 3/20/2026
Quantity: 90 tablet
Refill: No refills remaining

Prenatal MV-Min-Fe Fum-FA-DHA (PRENATAL 1 PO)

Instructions: Take by mouth.
Entered by: Roxanne Arenas, MA
Entered on: 8/19/2024

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F
Visit date: 4/15/2026

04/15/2026 - Postpartum Visit in SE MIDWIFERY

Clinical Notes

Progress Notes

Michelle Baas, CNM at 4/15/2026 0900

Author: Michelle Baas, CNM | Service: Obstetrics | Author Type: Midwife
Filed: 4/15/2026 10:10 AM | Encounter Date: 4/15/2026 | Status: Signed
Editor: Michelle Baas, CNM (Midwife)

Subjective

Paige Molinari is a 32 y.o. female who presents for a postpartum visit. She is 6 week postpartum following a Vaginal, Spontaneous Delivery

Pregnancy was complicated by
Problem List

Patient Active Problem List
Diagnosis
Patient Active Problem List
Diagnosis

- History of pre-eclampsia in prior pregnancy, currently pregnant in third trimester
- Short interval between pregnancies affecting pregnancy, antepartum
- Obesity in pregnancy, antepartum
- Mother currently breast-feeding (CMS/HCC)
- Elevated glucose level RESOLVED

Birth was complicated by None

Postpartum course has been complicated by nothing

Maternal readmission: no

Baby's course has been complicated by nothing

Newborn readmission: no

Baby is feeding by Exclusive breastfeeding at breast

Support at home: spouse and mother

Labor / hospital experience: admitted on 2/25/2026 at 37w4d for IOL for variable decels in triage, pt came in for concerns of LE swelling, hx of severe preE, found to have a borderline PCR as well in triage. Pt progressed to an uncomplicated NSVD, QBL 150 and 2nd degree repair. Uncomplicated postpartum course. On day of discharge, pt is breast feeding, pain is well controlled with PO analgesia, pt is afebrile, ambulatory, and desiring discharge to home.

Bleeding less flow than a normal period, a few small clots here and there

Patient is not sexually active. Contraception method is abstinence. Contraceptive plan None

ELR Grant and Dodge
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Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F
Visit date: 4/15/2026

04/15/2026 - Postpartum Visit in SE MIDWIFERY (continued)

Clinical Notes (continued)

Mood EPDS: 4 (4/15/2026 9:34 AM)
Edinburgh Postnatal Depression Scale: In the Past 7 Days
I have been able to laugh and see the funny side of things.: As much as I always could
I have looked forward with enjoyment to things.: As much as I ever did
I have blamed myself unnecessarily when things went wrong.: No, never
I have been anxious or worried for no good reason.: (1) Hardly ever
I have felt scared or panicky for no good reason.: No, not at all
Things have been getting on top of me.: No, I have been coping as well as ever
I have been so unhappy that I have had difficulty sleeping.: (1) Not very often
I have felt sad or miserable.: (1) Not very often
I have been so unhappy that I have been crying.: (1) Only occasionally
The thought of harming myself has occurred to me.: Never
Edinburgh Postnatal Depression Scale Total: 4

Pt denies concerns with depression and anxiety, however things have been challenging with her relationship with her brother. Life has been hard lately, Paige's husband is a contractor and he is remodeling their house, they have a newborn and now there is tension between Paige and her brother. She declines BH referral at this time, will reach out if needed. She started attended a mom and baby group at a coffee shop in Vail and that has been nice.

ROS
Pain: foot pain on left side, not sure what caused it but it has been this way since birth, doing foot soaks but it has not been helpful. Was also having headaches for a little while but they have resolved with Ibuprofen.
Stitches/Incision: healing well
Breasts: no breast or nursing problems
Bowel function: regular bowel movement
Bladder function: no problems with bladder

Objective
Vitals:

	04/15/26 0854
BP:	103/63
Pulse:	73
Resp:	18
Temp:	98.2 °F (36.8 °C)
SpO2:	98%

OBGyn Exam

Pap history:
Pap history: Last pap 5/17/22; due today

Assessment/Plan:
Paige Molinari is a 32 y.o. G3P1112 at 6 weeks postpartum.

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F
Visit date: 4/15/2026

04/15/2026 - Postpartum Visit in SE MIDWIFERY (continued)

Clinical Notes (continued)

Diagnoses and associated orders for this visit:

1. Routine postpartum follow-up (CMS/HCC)
2. Care and examination of lactating mother (CMS/HCC)
3. Pap smear for cervical cancer screening
   - TP w/ HPV if age 30-64(Q5yrs) Rflx HPV Genotype
4. Pelvic floor dysfunction in female
   - Referral to Physical Therapy (El Rio)
5. 6 weeks gestation of pregnancy

- Medical records from hospital encounter for delivery reviewed
- Follow up with El Rio MW in 1 year for WWE

Electronically signed by Michelle Baas, CNM at 4/15/2026 10:10 AM

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F
Visit date: 3/10/2026

03/10/2026 - Office Visit in SE MIDWIFERY

Clinical Notes

Progress Notes

Michelle Baas, CNM at 3/10/2026 1345

Author: Michelle Baas, CNM | Service: Obstetrics | Author Type: Midwife
Filed: 3/10/2026 10:58 PM | Encounter Date: 3/10/2026 | Status: Signed
Editor: Michelle Baas, CNM (Midwife)

Subjective

Paige Molinari is a 32 y.o. female who presents for a postpartum visit. She is 1 week postpartum following a Vaginal, Spontaneous Delivery

Pregnancy was complicated by
Patient Active Problem List
Diagnosis

- History of pre-eclampsia in prior pregnancy, currently pregnant in third trimester
- Short interval between pregnancies affecting pregnancy, antepartum
- Obesity in pregnancy, antepartum
- Mother currently breast-feeding (CMS/HCC)
- Elevated glucose level RESOLVED

Birth was complicated by None

Postpartum course has been complicated by nothing

Maternal readmission: no

Baby's course has been complicated by nothing

Newborn readmission: no

Baby is feeding by Exclusive breastfeeding at breast

Support at home: spouse and mother

Labor / hospital experience: admitted on 2/25/2026 at 37w4d for IOL for variable decels in triage, pt came in for concerns of LE swelling, hx of severe preE, found to have a borderline PCR as well in triage. Pt progressed to an uncomplicated NSVD, QBL 150 and 2nd degree repair. Uncomplicated postpartum course. On day of discharge, pt is breast feeding, pain is well controlled with PO analgesia, pt is afebrile, ambulatory, and desiring discharge to home.

Bleeding less flow than a normal period, a few small clots here and there

Patient is not sexually active. Contraception method is abstinence.

Mood: Pt denies concerns with depression and anxiety.

ROS
Pain: none now, but the "after pains" for the first 3 days were "horrific"

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F
Visit date: 3/10/2026

Clinical Notes (continued)

Stitches/Incision: 2nd degree repair, healing well, no discomfort
Breasts: no breast or nursing problems
Bowel function: regular bowel movement
Bladder function: no problems with bladder

Objective

Vitals:

	03/10/26 1323
BP:	116/67
Pulse:	77
Resp:	16
Temp:	98.1 °F (36.7 °C)
SpO2:	99%

Checking BP at home daily and they are all around 115/60.

Physical Exam
Constitutional:
General: She is not in acute distress.
Appearance: Normal appearance. She is not ill-appearing.
Genitourinary:
Breasts:
Right: No swelling, bleeding, inverted nipple, mass, nipple discharge, skin change or tenderness.
Left: No swelling, bleeding, inverted nipple, mass, nipple discharge, skin change or tenderness.
HENT:
Head: Normocephalic and atraumatic.
Mouth/Throat:
Mouth: Mucous membranes are moist.
Eyes:
Conjunctiva/sclera: Conjunctivae normal.
Neck:
Thyroid: No thyroid mass, thyromegaly or thyroid tenderness.
Trachea: Trachea normal.
Cardiovascular:
Rate and Rhythm: Normal rate and regular rhythm.
Heart sounds: Normal heart sounds.
Pulmonary:
Effort: Pulmonary effort is normal. No respiratory distress.
Breath sounds: Normal breath sounds. No wheezing.
Chest:
Chest wall: No tenderness.
Abdominal:
General: Abdomen is flat. There is no distension.
Palpations: Abdomen is soft.
Tenderness: There is no abdominal tenderness. There is no right CVA tenderness, left CVA tenderness or guarding.

03/10/2026 - Office Visit in SE MIDWIFERY (continued)

Clinical Notes (continued)

Comments: No diastasis recti
Fundus firm, midline @ 1 fb above SP, NT
Musculoskeletal:
General: Normal range of motion.
Cervical back: Normal range of motion. No edema, erythema or rigidity. No pain with movement.
Right lower leg: No edema.
Left lower leg: No edema.
Lymphadenopathy:
Cervical:
Right cervical: No superficial, deep or posterior cervical adenopathy.
Left cervical: No superficial, deep or posterior cervical adenopathy.
Upper Body:
Right upper body: No supraclavicular or axillary adenopathy.
Left upper body: No supraclavicular or axillary adenopathy.
Neurological:
General: No focal deficit present.
Mental Status: She is alert and oriented to person, place, and time.
Skin:
General: Skin is warm and dry.
Psychiatric:
Mood and Affect: Mood normal.
Behavior: Behavior normal.

Pap history:
Last pap 5/17/22; will be due at 6 wk PP visit, planning to do it at the VA

Assessment/Plan:
Paige Molinari is a 32 y.o. G3P1112 at 6 weeks postpartum.

Diagnoses and associated orders for this visit:
1. Routine postpartum follow-up (CMS/HCC)
2. Care and examination of lactating mother (CMS/HCC)

- Medical records from hospital encounter for delivery reviewed
- Follow up with El Rio MW in 5 weeks
- Ok to stop taking BP at home

Electronically signed by Michelle Baas, CNM at 3/10/2026 10:58 PM

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F
Visit date: 1/23/2026

01/23/2026 - Routine Prenatal in SE MIDWIFERY

Clinical Notes

Progress Notes

Maya Adams, CNM at 1/23/2026 1000

Author: Maya Adams, CNM | Service: Obstetrics | Author Type: Midwife
Filed: 1/23/2026 12:23 PM | Encounter Date: 1/23/2026 | Status: Signed
Editor: Maya Adams, CNM (Midwife)

Paige Molinari is a 32 y.o. G3P0111 at 32w5d who presents for routine prenatal visit.
Here with baby Audrey

Subjective

Feeling Well

Reports fatigue

Reviewed normal 3 hour GTT.
Objective

BP 121/80 (BP Location: Right arm, Patient Position: Sitting) | Pulse 110 | Temp 98.4 °F (36.9 °C) (Oral) | Wt 96.8 kg (213 lb 6.4 oz) | LMP 06/08/2025 (Exact Date) Comment: Currently breastfeeding, cycles returned at 4 months postpartum, 2 cycles before conception. | SpO2 96% | BMI 37.80 kg/m²

Fetal Heart Rate: 140 Fundal Height (cm): 32 cm at 32w5d

TWG: 13.3 kg (29 lb 6.4 oz)

Assessment/Plan

Problem List Items Addressed This Visit

Active Problems
History of pre-eclampsia in prior pregnancy, currently pregnant in third trimester
Obesity in pregnancy, antepartum - Primary
Other Visit Diagnoses
32 weeks gestation of pregnancy

EDUCATION provided today:
The following were addressed during this visit:

Education Trimester 2
- BWHC: eligibility for birth center birth
- Fetal movement monitoring

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01/23/2026 - Routine Prenatal in SE MIDWIFERY (continued)

Clinical Notes (continued)

Return to Clinic: 3 weeks, and PRN with any concerns or warning signs.
Recommended getting weekly appts set up today.

Electronically signed by Maya Adams, CNM at 1/23/2026 12:23 PM

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F
Visit date: 12/19/2025

12/19/2025 - Routine Prenatal in SE MIDWIFERY

Clinical Notes

Progress Notes

Maya Adams, CNM at 12/19/2025 1500

Author: Maya Adams, CNM | Service: Obstetrics | Author Type: Midwife
Filed: 12/19/2025 6:01 PM | Encounter Date: 12/19/2025 | Status: Signed
Editor: Maya Adams, CNM (Midwife)

Paige Molinari is a 32 y.o. G3P0111 at 27w5d who presents for routine prenatal visit.
Here with her daughter.

Subjective

Feeling Well

Reports no complaints

Reviewed elevated 1 hour results. She is aware, not sure about doing the 3 hour. Reviewed options to do fingersticks X2 weeks. She decided she will find time to do the 3 hour, reviewed instructions.

Objective

BP 125/79 (BP Location: Left arm, Patient Position: Sitting) | Pulse 101 | Temp 98.4 °F (36.9 °C) (Oral) | Wt 92.5 kg (204 lb) | LMP 06/08/2025 (Exact Date) Comment: Currently breastfeeding, cycles returned at 4 months postpartum, 2 cycles before conception. | SpO2 96% | BMI 36.14 kg/m²

Fetal Heart Rate: 140 Fundal Height (cm): 28 cm at 27w5d

TWG: 9.072 kg (20 lb)

Assessment/Plan

Problem List Items Addressed This Visit

Active Problems
Elevated glucose level - Primary
History of pre-eclampsia in prior pregnancy, currently pregnant in third trimester
Short interval between pregnancies affecting pregnancy, antepartum

EDUCATION provided today:
The following were addressed during this visit:

Education Trimester 2
- Lab and diagnostics reviewed

Return to Clinic: 4 weeks, and PRN with any concerns or warning signs.

12/19/2025 - Routine Prenatal in SE MIDWIFERY (continued)

Clinical Notes (continued)

Electronically signed by Maya Adams, CNM at 12/19/2025 6:01 PM

ELR Grant and Dodge
3655 E. Grant Road
TUCSON AZ 85716-2933

Molinari, Paige
MRN: 50196643, DOB: 12/13/1993, Legal Sex: F
Visit date: 11/21/2025

11/21/2025 - Routine Prenatal in SE MIDWIFERY

Clinical Notes

Progress Notes

Maya Adams, CNM at 11/21/2025 1130

Author: Maya Adams, CNM | Service: Obstetrics | Author Type: Midwife
Filed: 11/21/2025 12:17 PM | Encounter Date: 11/21/2025 | Status: Addendum
Editor: Maya Adams, CNM (Midwife)

Paige Molinari is a 31 y.o. G3P0111 at 23w5d who presents for routine prenatal visit.
Here with her daughter Audrey

Subjective

Feeling Well

Reports no complaints

Taking LDASA

Had FAS all normal but ltd views of spine, FOLLOW UP scheduled 12/5

Reviewed GDM screening for next visit or can do the week before, reviewed instructions, put orders in.

Objective

BP 115/74 (BP Location: Right arm, Patient Position: Sitting) | Pulse 90 | Temp 98 °F (36.7 °C) (Oral) | Resp 20 | Wt 90.3 kg (199 lb 1.6 oz) | LMP 06/08/2025 (Exact Date) Comment: Currently breastfeeding, cycles returned at 4 months postpartum, 2 cycles before conception. | SpO2 96% | BMI 35.27 kg/m²

Fetal Heart Rate: 140 Fundal Height (cm): 23 cm at 23w5d

TWG: 6.849 kg (15 lb 1.6 oz)

Assessment/Plan

Problem List Items Addressed This Visit

Active Problems
History of severe pre-eclampsia
Short interval between pregnancies affecting pregnancy, antepartum - Primary
Relevant Orders
GTT 1 Hour (Prenatal)
CBC and differential
RPR Screen w/rfx Titer & TPPA

Other Visit Diagnoses

23 weeks gestation of pregnancy

Relevant Orders

GTT 1 Hour (Prenatal)
CBC and differential
RPR Screen w/rfx Titer & TPPA

EDUCATION provided today:
No pregnancy checklist tasks were completed during this visit, and no tasks are pending completion.

Return to Clinic: 4 weeks or 5 due to holidays and has US in between visits, and PRN with any concerns or warning signs.

Electronically signed by Maya Adams, CNM at 11/21/2025 12:17 PM